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A child's right to circumcision

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Introduction

The circumcision of male children is central to Judaism (Genesis 17) and Islam; until recently it was also a common practice amongst most of the population of the UK, and remains very common in the USA (where current estimates are that ≈75% of men are circumcised). The reasons for this and for the decline of circumcision are beyond the scope of this article [1]. Non-therapeutic and non-consensual circumcision, when not mandated by religion and not justified by the needs of cultural identity, is not easy to defend, and none is offered here. Furthermore, the author maintains that female genital mutilation and male circumcision are radically different and that, even within a cultural pluralist framework which he defends [2], the former is unacceptable and the UK legislature was right to proscribe it [3] (but see [4]).

This article examines first the English law on male circumcision, in particular that which relates to consent, and second constructs a male child's rights to a ritual circumcision, using international instruments, primarily the United Nations Convention on the Rights of the Child. The author is committed to the need to take children's rights seriously [5], to recognize the moral status of children [6], to see children as persons rather than property, as participants in social processes rather than as objects of intervention or social problems [7]. To deny a Jewish or Muslim child a circumcision is to undermine that child's right to cultural heritage and identity. That there is necessarily an interference with bodily integrity does not mean that the child's right to bodily integrity is thereby infringed. Parents are only doing to their children what the overwhelming preponderance of them would wish done were they able to express a view. The ritual removal of a child's foreskin enhances that child's bodily integrity.

English law and male circumcision

Whether an adult male can give an effective consent to his own circumcision (e.g. after a conversion to Judaism or Islam) is not a matter upon which an English court has specifically pronounced. However, English law clearly permits gender reassignment (though not gender re-categorization [8]), cosmetic surgery (face-lifts, breast enlargement, cosmetic changes to a person's nose [9]) as it does ear-piercing and tattooing (although the tattooing of minors other than by or under the direction of a registered medical practitioner for medical reasons — and what medical reasons can there be? — is an offence [10], the implication being that the tattooing of adults must be lawful), and reasonable physical chastisement [11]. When, in the leading case of R. vs. Brown (1994 1 AC 212) (where a group of sadomasochists participated in acts of violence which included genital torture), the House of Lords held that a person's consent was no defence to a serious criminal assault. Lord Templeman, in an *obiter dictum*, specifically referred to ritual male circumcision as a lawful activity such that consent would be a good defence to a criminal prosecution. However, he also noted that what is regarded as lawful may change over time (e.g. duelling, and see [12]). It should follow that a doctor performing a male circumcision is unlikely to be guilty of a criminal offence or to be civilly actionable in tort. If English law were to regard consent to circumcision as invalid or as being against public policy, with the result that prosecution of a doctor might succeed, it is still unlikely that a civil action could be brought, unless of course the circumcision were negligently performed [13].

If any adult can consent to a circumcision upon himself, then so can a child of 16 years [14] provided a circumcision is to be regarded as 'treatment' (although 'treatment' has been interpreted narrowly, e.g. [15], where it was pointed out that otherwise Parliament would not have thought it necessary to provide in [14] that treatment includes diagnosis and ancillary procedures such as the administration of an anaesthetic). In Scotland, there is no doubt, because the legislation refers to 'procedure or treatment' [16] and circumcision is unquestionably a medical procedure, even if doubt hinges upon whether it is a medical treatment. A child under 16 may also be able to consent to circumcision when 'Gillick competent' (according to the test propounded by Lord Scarman in [17]; see also [18]). This is a capacity-related [19] rather than age- or status-related test and no age can, or should, be stipulated. However, it may be conjectured that comparatively young children may attain Gillick competence [20]. Most Jewish circumcisions take place on the eighth day of the boy's life and within Islam it is also recommended that

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it be performed on newborn boys (although in some communities it is done just before puberty). In most cases therefore, the question of a child's consent does not arise.

Can a parent consent to a son's circumcision? The power to consent to medical treatment is an aspect of parental responsibility [21]. However, the House of Lords has said that this must be exercised in accordance with the welfare principle [17], i.e. in the best interests of the child [21]. Kennedy and Grubb [22] note that 'inherent in the notion of ''best interests'' is that the medical intervention must be therapeutic, i.e. intended to benefit the particular individual', but they accept that the line between therapeutic or not is often difficult to discern [22]. What is in a child's best interests is to some extent conditioned by the context, which includes the religious context. If a parent's decision to circumcise were challenged, a court would apply the paramountcy principle (where circumcision is clearly a matter relating to a child's upbringing). It would be bound also to take account of the philosophy of current children's legislation [23] which, as the Lord Chancellor of the day wrote [24], emphasizes 'the integrity and independence of the family [as] the basic building block of a free and democratic society'. He stressed that 'the need to defend it should be clearly perceivable in the law'. As a result, the Children Act 1989 requires a child to be suffering or likely to suffer 'significant harm' [21] before coercive state intervention to interfere with child-rearing practice can be countenanced. Furthermore, as Kennedy and Grubb note, 'regard may, of course, be paid to a religious view of a proxy [that is, a parent] when the effect on the child is not significant (e.g. male circumcision)' [22].

It has to be accepted that 'best interests' is an indeterminate notion [25]. English courts, even in politically sensitive areas such as sterilization, have rarely seen the conjunction of best interests and human rights (in striking contrast to other common law jurisdictions, compare [26] and [27]). It can be maintained that cultural identity, a sense of belonging to a religious and cultural group, is a fundamental human right. There is international authority to support this; the International Covenant on Economic, Social and Cultural Rights (1967) provides that 'all peoples have the right of self-determination. By virtue of that right they freely determine their political status and freely pursue their social, economic and cultural development'. This does not mean that every religious practice can be tolerated in the name of multiculturalism [28]. There is a balancing exercise to be undertaken to determine whether a particular procedure or treatment is in a child's best interests. The relative harms and benefits of ritual male circumcision are such that a parent's decision to circumcise in the name of religion should not be questioned. More difficult questions could arise where parents disagree, and even more difficult questions are posed where the child has mixed parentage (perhaps a Jewish mother and a Gentile father). Where two Jewish or Muslim parents disagree, a specific issue order [21, sec. 8(1)] could be sought; the paramountcy test would be applicable [21, sec. 1(1)] and the welfare checklist which includes the child's 'background' [21, sec. 1(3)(d)] would become relevant, as would any 'harm' (not in this instance significant harm) which the child was 'at risk of suffering' [21, sec. 1(3)(e)]. As harm relative to benefit is small, a court, it is argued, should authorize a circumcision. In cases of mixed parentage, such a conclusion would be less justifiable, *a fortiori* where the child would not be regarded by the religion concerned as of that religion (notably the child of a Jewish father and a Gentile mother).

It has been assumed that the decision to circumcise can be taken on religious and cultural grounds by a parent. However, the courts have held that certain decisions, notably non-therapeutic sterilization of children with learning disabilities [26] and the turning-off of life-support systems where a child is in a persistent vegetative state [29,30], can only be taken by courts using *parens patriae* jurisdiction and applying the criterion of best interests. An Australian court adopted the same view about gender reassignment [31]. Some have argued [32] that male circumcision is in the same category of decision ('major', 'severe', 'irreversible'), but this cannot possibly be so. It is routine, not major, takes only a few minutes, need not be performed in hospital, causes discomfort (and this fleeting) rather than pain (although this is controversial [33,34]), and may not be irreversible (foreskin restoration is usually possible, see Brandes and McAninch, this supplement). Furthermore, it would be a waste of court resources (unless there were to be one test case).

If the courts were to hold that a parent cannot consent to a child's circumcision either because it is not in the child's best interests or because the decision is beyond the authority of a parent and has to be taken by a court (and, it is submitted, both of these conclusions are unlikely to be reached) the question arises as to the potential liability of doctors and parents. If the parental consent were invalid, the circumcision could constitute the tort of battery and the crime of occasioning actual bodily harm [35]. The child would have to sue through a 'next friend' [36] or could conceivably wait until arriving at the age of majority to sue (the limitation period would only begin to run on majority) [37]. It has been argued [38] that a doctor if sued would be safe if she/he acted in accordance with the then current standard of responsible medical opinion [39]. If a stricter test than this were applied, doctors would be reluctant to perform several procedures on children which are commonly performed on legally

consenting adults, as adults need not necessarily act in their best interests. The application of the responsible medical-opinion test (the so-called Bolam standard) should exonerate doctors from liability for performing a circumcision because, even if there is a groundswell of medical opinion against the practice, there is still a reasonable body of medical opinion that supports it. (The Australian College of Paediatrics recently stated that 'it is not possible to be dogmatic on the exact risk/benefit ratio' [40,41].)

Circumcision: a child's right

It has been maintained in the first half of this article that ritual male circumcision is lawful in the UK. The argument in the second half is that it should remain lawful. Despite arguments to the contrary [1,42], it is not child abuse. Government guidance describes male circumcision as 'simple and straightforward [and] pos[ing] no harm or danger to the health and welfare of the child' [43]. English law defines abuse in terms of 'significant harm' [44], but 'significant' has to be situated within relationships [45]. It is necessary to examine a specific child in the context of a specific home; abuse in one context is not necessarily abuse in another. Context also includes culture [46]. The harm has to be attributable to the care given to the child not being what it would be reasonable to expect a parent to give to him. It is not reasonable to expect a parent to have a male baby circumcised, but it would be unreasonable to expect a Jewish parent not to do so. Far from ritual circumcision constituting abuse, it is arguable that the failure by Jewish or Muslim parents to do so is abuse, the child being likely to suffer significant harm.

The United Nations Convention on the Rights of the Child, which the UK has ratified, defines abuse more extensively than the English Children Act. Under Article 19, States Parties are to take 'all appropriate legislative, administrative, social and educational measures to protect the child from all forms of physical or mental violence, injury or abuse, neglect or negligent treatment, maltreatment or exploitation, including sexual abuse'. Ritual male circumcision is not an act of violence. To suggest otherwise is both to distort language and to stand outside the critical reflective attitudes of the participants in the activity [47]. It does not cause injury, as this is understood in the Convention, is not abuse, and certainly not sexual abuse, for reasons which have already been given. The United Nations Committee on the Rights of the Child found the UK to be in breach of Article 19, but not because it allowed ritual male circumcision; it indicted corporal punishment practices [48].

Opponents of ritual male circumcision may also point to Article 24(3) of the Convention. This mandates States Parties to 'take all effective and appropriate measures with a view to abolishing traditional practices prejudicial to the health of children'. The target of this provision is female genital mutilation [48]. Delegates from the UK, the USA and Canada would have preferred outright condemnation of this practice. That there is a general formula is the product of diplomatic compromise. As far as male circumcision is concerned, medical opinion is sufficiently divided for it to be possible to conclude that the practice is prejudicial to the health of male babies. However, those who cite this Article to attack circumcision rarely discuss the preamble in the Convention which refers to 'the importance of the traditions and cultural values of each people for the protection and harmonious development of the child'.

Far from ritual male circumcision constituting abuse or a prejudicial traditional practice, or a threat to a child's bodily integrity, it is argued here that male Jewish and Muslim babies have the right to be circumcised. The argument that the decision should be taken by the person himself when he is old enough to consent is unconvincing, if not self-defeating. It is not clear what 'old enough' means; it must be presumed this occurs when the boy is Gillick competent [17].

However, it would not be surprising if opponents of circumcision were to hold that willingness to undergo what would be then be a painful procedure was an indication that the child was not so competent. If the intention is to prevent the infliction of pain, as opponents of male circumcision argue, then why only allow it at the time when the pain and other trauma will be greater? Is not this to turn consent into some kind of fetish? To circumcise a baby is hardly to interfere with his autonomy; nor, obviously, can he consent. However, what is possible is what has been called 'future-oriented consent' [49]. Adults can choose for children not in terms of what they then want, but in terms of maximizing those interests that will make it possible for them to develop life plans of their own. Sending a child to school is the most obvious example. It may be suggested that initiating a child into his cultural heritage and religious destiny is another. The question is: can the decision to circumcise at a time when the child cannot consent but also will suffer very little be justified in terms that the child would eventually come to appreciate? Looking back as an older child or an adult, would he appreciate and accept what was done to him? A few do not, but the vast majority of adult male Jews and Muslims do. To them, cultural and religious identity, the sense of belonging to a group, is of greater significance than minor invasive treatment administered when they were unaware of it.

The United Nations Convention on the Rights of the Child supports this right. Thus under Article 8, 'States Parties undertake to respect the right of the child to

preserve his or her identity'. 'Identity' is not defined in the Convention but, although the Article is largely targeted at Argentina's 'Dirty War', when children disappeared and/or were illegally adopted [49], there is no reason why such a limited interpretation should be put on the provision. Denial of circumcision would constitute an undermining of religious and cultural identity.

There is also Article 14: under this, 'States Parties shall respect the right of the child to freedom of thought, conscience and religion', and 'shall respect the rights and duties of parents ... to provide direction to the child in the exercise of his or her right in a manner consistent with the evolving capacities of the child'. Furthermore, 'Freedom to manifest one's religion or beliefs may be subject only to such limitation as are prescribed by law and are necessary to protect public safety, order, health or morals, or the fundamental rights and freedoms of others'.

To deny a Jewish or Muslim child a circumcision removes from him the ability fully to participate in the religious life of his community and as such undermines his freedom of religion. The act of ritually circumcising a child is a constituent element of the 'direction' referred to in paragraph 2. Moreover, circumcision does not infringe any of the stated limitations in the third paragraph of Article 14.

It is unlikely that Parliament will wish to criminalize or otherwise render unlawful the process of male circumcision. If it chose to do so, it would probably distinguish ritual circumcision from routine circumcision (which is anyway now an uncommon practice in the UK). However, should it consider doing so, it would need to examine another article in the Convention, Article 3. This directs 'In all actions concerning children, whether undertaken by public or private social welfare institutions, courts of law, administrative authorities or legislative bodies, the best interests of the child shall be a primary consideration'.

Much could be said about this overarching mandate but two observations suffice; 'best interests' does not exist in a vacuum and must accordingly be situated in its cultural context. The best interests of Jewish and Muslim boys may be abstracted from the generality of children. Ritual circumcision is in their best interests. Second, 'best interests' should not be looked at in the short-term only; a longer term perspective must be taken. Long-term benefit will invariably override transient trauma.

So far as is known, only Antiochus IV Epiphanes [50,51] (and Maccabeus 1:48, 2:46; 6:10), Hadrian [52], Stalin [53] and Hitler [54,55] have outlawed ritual male circumcision. Is this the company with which today's opponents of circumcision would feel happy?

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